SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Carboplatin AUC 4 and fluorouracil with radiotherapy

Head and neck: chemoradiotherapy · NCCP 00591 · version 4 · 21-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
Chemotherapy is given with radiation to improve control of cancer in the head and neck. Your team confirms the treatment aim and why carboplatin is suitable for your circumstances.

Schedule
Carboplatin AUC 4 on Day 1, with fluorouracil 600 mg/m²/day on Days 1–4, every 21 days for 3 cycles, only concurrently with radiotherapy.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ DPD result and severe fluoropyrimidine risks reviewed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Carboplatin
PLATINUM CHEMOTHERAPY

- ☐ Low blood counts and serious infection
- ☐ Low platelets and bleeding
- ☐ Anaemia and fatigue
- ☐ Nausea and vomiting
- ☐ Infusion or allergic reactions
- ☐ Kidney, nerve or hearing effects

Fluorouracil
ANTIMETABOLITE CHEMOTHERAPY

- ☐ Low blood counts and serious infection
- ☐ Mouth and throat soreness
- ☐ Diarrhoea and dehydration
- ☐ Painful palms or soles
- ☐ Heart problems including chest pain
- ☐ Rare severe neurological or early excessive toxicity

Discuss treatment aim and alternatives, expected benefit and uncertainty, all listed drug risks, infection and emergency advice, blood and organ checks, support medicines, fertility and contraception, and the option of declining treatment. Discuss radiation-specific acute and late risks separately with radiation oncology, and DPD testing, infusion/line care and the pump schedule.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00591 v4.

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